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World Health Organization (WHO)

Background Guide

TritonMUN XXXI – April 12th, 2026

MODEL UNITED NATIONS AT UNIVERSITY OF CALIFORNIA, SAN DIEGO

Table of Contents

Head Chair Letter

Dear Delegates,

My name is Ryan Lay and I am delighted to be your head chair for the 2026 WHO committee. I am from Cerritos and am a freshman majoring in Human Biology, and this will be my fifth year in MUN. In my opinion, MUN is a great opportunity to not only build on your existing leadership and diplomacy skills, but to also have fun and network with the delegates around you.

More about me, throughout my time in MUN, I uncovered a lot of topics revolving around the issues with the healthcare system. Outside of MUN, I have four white, typical Asian dogs that I am unable to see since I live two hours away. In my freetime, I really enjoy drawing on my tablet and playing mahjong or cards with my friends at Marshall. No shade to Eighth College, but I walk to Marshall everyday since my friends all reside there and their dorms are a lot nicer. Call me a nerd, but during my passing period, I really enjoy going to my lab and focusing on my projects. After my classes, I sadly only have two options: go take a "short" nap or study until my next class. For my future goals, I am a pre-med student and am 100% committed, so I hope to get into Medical School during my third year.

If you have any questions about MUN, the committee, or just general stuff outside of diplomacy, feel free to reach out. I hope to see creative hooks and unique solutions towards solving this very persistent crisis. Don't hesitate to contact me for questions. Anyways, have a fun time in committee, looking forward to seeing you all debate and collaborating with each other!

Sincerely,
Ryan Lay
rhlay@ucsd.edu

VICE CHAIR LETTER

Dear Delegates,

My name is Eva Christy and I am excited to be serving as your vice chair in this WHO committee. I am a second year Environmental Policy student from Long Beach, California, and I have been doing MUN for five years!

A little bit about me: I spend my free time crocheting, playing tennis, and cooking. The last thing I cooked was a tofu, veggie, and rice dish, and the last thing I baked was a lemon raspberry cake. I typically cook to a soundtrack of Charli XCX, Zara Larsson, and Tate McRae. If I'm not in the mood for music, I often listen to news podcasts or the newest addition to my podcast library, a technology podcast called *Hard Fork*. I know nothing about technology, so that has been fascinating for me. I also work at UCSD's library and enjoy reading when I can get a free second of time.

If you have any questions related to MUN or anything else, feel free to reach out to me. I'm looking forward to meeting all of you and to having a diplomatic, engaging, and fun committee!

Sincerely,
Eva Christy
echristy@ucsd.edu

Position Paper Guidelines

TritonMUN Position Paper Guidelines

- Position Papers are due on **April 3rd at 11:59pm.**
  - Requests for extensions must be sent by the advisor to TritonMUN by **March 22nd.**
- Position Papers must be submitted to the **Google Form** linked here:
  - https://forms.gle/378QG1eG6Tw2t4f28
- Format.
  - Papers should be double-spaced in Times New Roman 12 pt. font and include no pictures in a justified format.
  - At the top left, include your country's official name, committee, and topic.
  - Please name your papers **"WHO - [Country Name]"**
  - Required* Sections: Each of these sections should be clearly labeled (minimum 1 page & maximum 2 pages)
    - Background
    - UN Involvement
    - Country Position
    - Possible Solutions
    - Works Cited (MLA Format)

*If you have any questions on position papers, email rhlay@ucsd.edu

*Some committees may ask for different formatting or an alternative to a position paper altogether, this will be clearly noted in the background guide for that committee

Rules of Procedure

Technology Policy:

In this WHO committee, technology is viewed as a helpful tool that can support debate. Delegates are encouraged to utilize their laptops and devices throughout committee, including unmoderated and moderated caucuses, to help memorize their speeches and refamiliarize themselves with their information. Technology will mainly be used to draft working papers and resolutions, but delegates are not allowed to conduct further research than they have during the conference.

Cell phones and distracting devices are highly discouraged in committee and any improper usage of technology, whether it is distracting yourself or the delegates around you, will result in a reduction in diplomacy. More importantly, TritonMUN has a strict no-AI policy; any and all usage of ChatGPT or other AI platforms will result in a disqualification in committee awards if caught during conference or a disqualification in research awards if found in delegate's position paper and will be brought up with their respective advisor.

Dei and AI Policies

Sensitivity & Harassment Policy:

TritonMUN is committed to fostering a healthy environment for diplomacy and dialogue, and upholding standards of diversity, equity, and inclusion for all delegates. Harassment or disruptive behavior of any kind—whether verbal or non-verbal, in or out of committee—is strictly prohibited. All attendees and staffers should be treated with equal respect, and discrimination on the basis of race, color, gender identity, sexual orientation, religion, nationality, ethnicity, or physical/mental disability will not be tolerated. This list is not exhaustive, and should not be treated as such. **Please report any instances of harassment to conference staff, either directly or through the Anonymous Feedback Form.**

In addition, we acknowledge that topics discussed in-committee may be politically sensitive, particularly in Specialized and Crisis Committees. While we understand that delegates may be immersed in the action and setting of their committee, delegates must maintain decorum and refrain from inflammatory language and actions, even when doing so may be historically accurate to one's character, group, or country. When drafting crisis directives, delegates should refrain from pursuing any action that explicitly violates human rights, including—but not limited to—enslavement and genocide. **Please refer to your committee's background guide for specific sensitivity statements and guidelines, and direct any questions to the committee's head chair.**

All attendants should observe these policies during the entirety of the conference, even outside formal debate, including opening/closing ceremonies, "FUNMUN", etc. Please note that violation of these policies may result in consequences such as disqualification from awards and contact towards the delegate's Advisor. At TritonMUN, our goal has always been focused on the delegate's abilities to enjoy and have fun in committee. However, in order to truly enjoy TritonMUN, delegates must feel safe in committee, which starts with respecting one another.

Plagiarism & Generative AI Policy:

TritonMUN recognizes that Model United Nations provides a space for aspiring leaders to showcase their work and abilities, and that all delegates wish to be evaluated in a fair and accurate manner. As such, we expect all delegates to perform with integrity and refrain from plagiarizing the work of their peers. This includes—but is not limited to—claiming the work of another delegate as one's own, including another bloc's unique clause without prior approval from the original writer, and replacing another's work with one's own (i.e. overwriting).

In addition, delegates are strictly prohibited from utilizing generative AI to produce position papers, resolutions, and directives, in part or in their entirety.

Resolution Work Outside of Committee:

TritonMUN wishes that all delegates use their breaks and downtime as intended: to connect, relax, and refresh. While not in committee, delegates are strictly prohibited from working on resolutions and directives. In addition, this means that pre-writing, or preparing resolutions and directives before the start of the conference, is strictly prohibited.

Technology Policy:

Delegates should expect the use of technology to be limited during committee sessions. Instead of having a universal technology policy for TritonMUN, such policies are individualized for each committee and set by their daises. Please refer to each committee's background guide to understand their technology policies, and direct any questions to the committee's head chair. Repercussive actions will be taken at the discretion of the head chair.

Key Terms

1. **Antimicrobial Resistance (AMR):** occurs when microorganisms evolve so that medicines designed to kill them no longer work and are ineffective.
2. **Antibiotics:** medicines used to prevent or stop the growth of bacteria, but are ineffective against viral infections like the flu.
3. **Microorganism:** microscopic living organisms such as bacteria, viruses, fungi, or parasites.
4. **Pathogen:** microorganism that causes disease in humans, animals, or plants.
5. **Superbug:** bacteria strain that has become resistant to multiple antibiotics through multiple exposures, making antibiotics or other medications impossible to treat.
6. **Bacteria:** single-celled microorganisms found everywhere, some of which cause infections and diseases.
7. **Virus:** tiny infectious agent that reproduces inside host cells and cannot be treated with antibiotics.
8. **Mutation:** change in genetic material that can allow bacteria to survive antibiotic treatment and pass resistance to future generations.
9. **Antimicrobial Stewardship:** coordinated efforts to ensure antibiotics are prescribed and used responsibly to slow resistance.
10. **Over-the-Counter (OTC) Sales:** medications sold without a prescription.
11. **Overprescription:** antibiotics are prescribed unnecessarily or too frequently.
12. **Livestock Antibiotic Use:** giving antibiotics to farm animals and livestock for disease prevention or growth promotion.
13. **Low- and Middle-Income Countries (LMICs):** countries with developing economies that often face limited healthcare infrastructure and regulatory capacity.
14. **Disease Hotspot:** geographic area with especially high levels of antimicrobial resistance.
15. **Sustainable Development Goals (SDGs):** 17 United Nations goals aimed at promoting global health, economic development, and sustainability by 2030.

Addressing the Escalation of Antimicrobial Resistance in South Asia

Background

Introduction

Antibiotics have revolutionized the medical industry since their introduction in the early 20th-century because they have enabled doctors to treat deadly diseases and infections while driving progress in surgical techniques and contemporary medical practices. The use of antibiotics without proper prescription guidelines has resulted in higher rates of antimicrobial resistance (AMR) throughout Southern Asia. This has led to the decline of quality healthcare, resulting in an alarming rise in mortality rate, causing over 1.27 million deaths in 2019 alone out of the 4.7 million deaths globally.

Overall, AMR occurs when germs, which include bacteria, fungi, and viruses, are not affected by antibiotic drugs. This effect can lead to extra funds being allocated for the creation of new drugs and medication production within the Research and Development (R&D) Industry. AMR mostly happens when a few bacteria, after being exposed to antibiotics, survive, mutating and passing on the resistant gene to the next generations. This is mainly because the reproduction cycle of the pathogen is extremely short, meaning that the number of offspring is large. Due to the prevalence of AMR, numerous drug companies have suffered from increased costs of creating antibiotic drugs while earning very little for their efforts, creating an equal balance between the research and financial sectors. AMR has also increased the danger associated with surgeries, cesarean births, cancer chemotherapy, hip replacements, organ transplants, and many other procedures that require open wounds. This inefficient way of creating drugs is made even worse by the inefficient use of antimicrobials and the increased rate of drugs used in livestock. Based on all of these difficulties, based on research from the World Bank, over 1 trillion USD will be added as a whole to the medical bill by 2050. The problem caused by AMR is extremely prevalent in this world, endangering the citizens through resistant diseases while skyrocketing the financial struggles on a global aspect.

History

The very first antibiotics that were developed originated from ancient civilizations, which comprised antiseptic properties from plant life and fungus. Nevertheless, this term was introduced in the 1950s by Selman Waksman, a scientist who developed streptomycin for the treatment of tuberculosis by using certain microscopic organisms and filtering antibacterial properties. Following this, penicillin was created in 1928, which led to the use of pharmacotherapy treatments. Immediately after the discovery, a case of AMR was noted for sulfonamides, while penicillinase was noted before the use of penicillin itself. Currently, AMR has been as prevalent as ever in South Asia, especially in India and Bangladesh.

Problems in Southern Asia

At this point, the Southern Asian region has eight countries: Afghanistan, Bangladesh, Bhutan, India, Maldives, Nepal, Pakistan and Sri Lanka. The World Health Organization (WHO) states there have been almost 389 thousand deaths linked to AMR in South Asian countries. Additionally, people are finding AMR cases in freshwater sources in places like eastern Nepal. In India they are also found on the coasts of Kerala, Tamil Nadu and Andhra Pradesh, as well as the Kelani and Gin rivers in Sri Lanka.

The idea of Antibiotic Resistance is a very prominent problem but the people who are most affected are those who live in very poor areas in Southern Asia where citizens do not have a lot of money. In Southern Asia, many areas have a lot of farmers that raise livestock and animals like cows and chickens. Currently, the livestock industry is booming, with numbers of these animals having gone up by more than 100 percent in the last thirty years. Many people in Southern Asia are facing huge pressure to grow an extreme amount of crops that must contain good quality in a short amount of time. This means they have to use a more modern way of farming, which includes treating animals with antibiotics for the purpose of producing more meat. There are two reasons why AMR is a big problem in Southern Asia. Citizens who live in the countryside are trying to identify their problems and are taking medicine on their own without the permission of their provider. Because of this, many are not taking the correct dosage of medicine or are not using it as required. People in these areas also truly believe that antimicrobial drugs can cure almost any disease, including those beyond its intended scope. They are unaware that these drugs do not work for their common health problems and that they are ineffective in curing viral infections and other diseases. This is because they do not know about the unintended consequences of using these drugs in excess. They know that these drugs are important and make them feel secure and safe, and therefore they consult doctors and demand antibiotics.

When we consider the case of healthcare in Southern Asia, we find that there are three major issues that have led to antimicrobial resistance. Firstly, people have the misconception that antibiotics can cure anything and do not have much information regarding resistance and the use of antimicrobials. Secondly, healthcare practitioners may use antibiotics as a quick way to solve many problems in the healthcare sector. This may be done without waiting for lab results to come in and, in some cases, without the need to write prescriptions. This is mainly due to the fact that healthcare practitioners may not always receive adequate training on the use of antibiotics and may feel that providing antibiotics is the right thing to do in the first place. Thirdly, the pharmaceutical industry may offer incentives to encourage the sale of antibiotics. This problem also exists in the policies and their implementation. It has to be noted that in some countries like Bhutan and Nepal, a prescription is needed to procure antibiotics. However, such regulations are not strictly adhered to in India and Pakistan. It is not just the formulation of policies that is a problem, it is the implementation as well. There are also the issues of quality care and financial rewards that can cause the abuse of antibiotics. Thus, there is a need to provide education to health practitioners about the responsible use of antibiotics. Generally speaking, Antimicrobial Resistance (AMR) is a global problem. However, currently, South Asia is experiencing some of the worst antimicrobial resistance patterns. Rural communities as well as communities experiencing poor water sanitation in India, Nepal, and Sri Lanka live with the problem of AMR. The situation is becoming worse as a result of excessive use of chemicals in farming, people taking antibiotics without a doctor’s advice, lack of proper regulations on controlling the use of antibiotics, as well as a lack of proper antimicrobial management systems. This is a challenge for the healthcare system as it is leading to many deaths.

Delegates should ensure they work towards improving prescription control as well as sustainable farming practices. They should also consider what they as individuals, as an economy, and as a government can do about the situation of AMR in South Asia. The situation of AMR is a crisis situation that demands all the parties involved to work together towards finding a solution. AMR is a major threat to public health in South Asia, which demands a combined effort towards ensuring it does not spread further, saving both South Asia and the world from these resistant diseases.

United Nations Involvement

The first large-scale national response to the overlying issue of Antimicrobial Resistance (AMR) in South Asia began through the adoption of the Jaipur Declaration on AMR signed by the 11 member states of the World Health Organization (WHO) South-East Asia Region (SEAR). Understanding the fact that "the world is on the brink of losing its miracle cures," the WHO Director Margaret Chan began to publicize the pressing threat of the irrational usage of antibiotics. Due to the Jaipur Declaration, Health Ministers in South Asia pledged to enhance legislation to continue the effectiveness of antibiotics and promote policies that go against counterfeit antimicrobials, while also building multisectoral cooperation with both the private and public sectors. It was clear that AMR posed a threat to the gains made in the fight against HIV/AIDS, tuberculosis, malaria, and other infectious diseases, establishing the precedent that resistance is not just a health problem but a development and economic crisis for the region.

This response has been additionally advanced with the implementation of the Global Action Plan on AMR in 2015 by WHO in collaboration with the Food and Agricultural Organization (FAO). This action plan sets five strategic objectives to combat AMR: raising awareness, improving surveillance, reducing infections, controlling usage of antibiotics, and providing sustainable funding for innovation and research. Upon September 2016, this process reached a historic milestone with the UN General Assembly meeting, which was the fourth-time a healthcare problem has been identified in this conference. The meeting has been a landmark event with the member states allowing for the development of National Action Plans (NAPs) to help strengthen the management of antimicrobials and vaccines.

The WHO Regional Office for South-East Asia has undertaken structured situational analysis to monitor the progress of AMR control since the 2016 political declaration. Three self-assessment surveys were conducted during 2016 to 2018, assessing 31 indicators under eight focus areas. All 11 SEAR countries have developed and endorsed their NAPs in accordance with the Global Action Plan, with eight Member States updating their NAPs since 2022. Throughout this time, improvements with national involvement began to steadily increase with implementation levels rising to 40% by 2018 and to 64% by 2021. Although around 90% of countries added in the NAPs initiated this implementation, only 27% of them monitored progress, meaning that merely a small amount was shown to not consistently maintain this plan. Additionally, no country recorded regression in implementation status, which could be a result of insufficient incentives and reduced funding measures. However, surveillance capacity was massively improved with 10 out of 11 South Asian countries initiating AMR security within the human sector. Disparities began showing up with progress being slowed within the animal and livestock sectors; meanwhile, the environmental sector leads with the lowest progress due to the underwhelming education and warning system for AMR being underdeveloped.

Recognizing that AMR is not only against human medicine, the Regional Tripartite Coordination between WHO, FAO, and the World Organisation for Animal Health (WOAH) expanded in 2022 to add in the United Nations Environmental Programme (UNEP) to form a "Quadripartite." This finalized form formalized the One Health Approach to fight against AMR. Then, in 2021, the Third One Health Situational Analysis highlighted the developments that were required to stabilize the issues in the realms of governance, surveillance, antimicrobial stewardship (AMS), infection prevention and control (IPC), and future coordinated movements. Despite these new measures, the transparency regarding the environmental problems and the early warning systems were still in their initial stages. Antimicrobial residues in the soil and water, the misuse of antimicrobials in agriculture, and the improper disposal of waste have all continued to contribute towards the development of AMR.

Currently, the latest development within the United Nations was brought about by UN Resolution A/RES/79/2 in October 2024, acknowledging AMR as a serious issue and a threat to human, animal, and agricultural life, while also impacting food security and financial systems. This resolution urges all member states in WHO to have their multisectoral NAPs updated and funded in coordination with neighboring countries, and is predicted to be implemented by 2030. Furthermore, it also commits to advancing sustainable economic growth hoping to mobilize over 100 million USD to ensure that around 60% of nations have the acceptable funding for NAPs. They also stress the need for an equal distribution of antimicrobials and diagnostics in low-income areas within Southern Asia.

Case study: New Delhi, Northern India

Antimicrobial Resistance (AMR) is a heavily prevalent issue especially in New Delhi, Northern India. Due to pollution and a high density of population of nearly 16.9 thousand citizens every square kilometer, New Delhi is a global hotspot for AMR, or more specifically, the gene NDM-1. NDM-1 was initially identified in a patient located in New Delhi's hospital in 2008 and was causing a drug-resistant form of Klebsiella pneumoniae, a bacteria that is usually harmless but can be dangerous if it starts an infection. Shortly later, the same AMR gene was identified to be infecting New Delhi's water supply and seepage areas, indicating that human waste and hospital discharge are affecting the rise of contamination. Currently, the NDM-1 gene is found in more than 100 countries worldwide due to international travel and global trade, meaning that although diseases can be centered around Southern Asia, it is still an issue for the whole world.

To identify the root cause of the formation of resistance within New Delhi, researchers undertook a substantial environmental investigation in 2014. These researchers collected water samples during the winter and summer months from 12 hospitals, 12 Sewage Treatment Plants (STPs), 20 sewer drains, and 5 sites along the Yamuna River, which receives a substantial amount of wastewater from the city. Their investigation underscored the quantity and quality of bacteria that are related to fecal contamination and resistance genes. Moreover, their research revealed that the majority of AMR was located within hospital regions during both seasons. In the summer, hospital waste was found to have at least ten times more resistant bacteria and a hundred times more of the important resistance gene than other sites, concluding that hospital wastage is a major problem when dealing with AMR.

Recently, the general situation in Northern India is characterized by numerous issues in the country's environment and healthcare system. This includes lack of effective antimicrobial stewardship (AMS) programs, over-the-counter sales of antibiotics, and the usage of old prescriptions. According to the WHO, nearly 67% of antibiotics are being bought without prescriptions in New Delhi's private pharmacies, while 44% are being prescribed with incorrect respiratory infections that can lead to AMR. Today, the Indian Council of Medical Research has been shown to develop up-to-date regional treatments guidelines that promote the proper use of antibiotics in a responsible manner. Laboratories will also be encouraged to undergo tests before treatment and utilize narrow-spectrum antimicrobials whenever available.

However, a gap between knowledge and hands-on practice continues to exist in India. Until Northern India consistently enacts stronger regulations and better waste management practices, especially near hospitals and medical centers, AMR in New Delhi and adjacent cities will resume to pose a risk to global health and national development.

Case Study: Dhaka Division, Urban Dhaka City

One of the major hotspots for the spread of antimicrobial resistance (AMR) in South Asia is urban Dhaka, the capital area of the Dhaka Division. Typhoid fever, a disease caused by Salmonella enterica serovar Typhi (a stain of bacteria), is the main concern in this area. Typhi is an extremely common human-restricted pathogen that can be spread through food and water. However, with an incidence rate of between 292 and 395 cases per 100,000 people annually, typhoid fever is a serious public health issue in Bangladesh.

A study researched the genomic surveillance of 202 isolates from 2004 to 2016 within three urban sites in Dhaka was conducted. This case study identified the genetic diversity and identified 17 types of genotypes that have been very evident in the country. The most dominant genotype was genotype 4.3.1, or H58. H58 is also identified globally and spreading across South Asia and Africa. However, there was evidence of unique patterns in Bangladesh, such as the identification of novel local subclades that were largely restricted to the Dhaka region.

The 1970s were a time when doctors discovered a type of typhoid disease that does not respond to medicines like ampicillin, chloramphenicol, and trimethoprim-sulfamethoxazole. This typhoid disease is called drug-resistant typhoid disease. Typhoid disease became resistant to fluoroquinolones, which is a type of antibiotic that people use a lot. In the city of Dhaka, something interesting happened starting in 2012. Doctors noticed that the old antibiotic-resistant typhoid disease strains, which are called H58 strains, were being replaced by typhoid disease strains that had changes in them and these new strains were not the usual H58 strains of drug-resistant typhoid disease.

A KAP survey of 704 parents in urban Dhaka found disturbing findings. While over 80% of the parents trusted doctors' prescriptions and disagreed with unrestricted use of antibiotics, 58% of the parents reported using antibiotics on their children without consulting a doctor. Over 63% of the parents believed that antibiotics would improve fever and the common cold, which are viral diseases. Additionally, 27% of the parents used antibiotics to treat similar diseases, and 26% of the parents discontinued antibiotics after the symptoms of the infection had improved. Only 36% of the parents exhibited good antibiotic use behavior.

The case of Dhaka has shown a two-pronged crisis of genetically evolving resistant S. Typhi strains, as well as antibiotic use behavior in the community setting. Improved surveillance, better sanitation, and regulation of antibiotic use is the way forward to address the crisis of antibiotic use in the community setting of Dhaka. Otherwise, the region of Dhaka would become a global exporter of resistant typhoid strains, which would have severe health consequences for the global population.

Questions to Consider

1. How are South Asian countries affecting the global issue of AMR as a whole?
2. What are the root issues that stem from AMR specially in Southern Asia? Why are countries not consistently following their NAPs? Are there incentives that countries can receive?
3. Are there any economical issues that countries are facing when providing and encouraging proper usage of antibiotics?
4. What are the issues regarding the education on the over usage of medications? Why are citizens in South Asia unaware of AMR?
5. What are some existing frameworks or policies that have worked in the past that reduced the cases of AMR? Can they be implemented in South Asia as well?
6. Are there any other alternatives to antibiotics that can slow the bacteria from mutating?
7. How does the healthcare system in low-income areas within South Asia operate? Do they promote or obstruct the spread of AMR?
8. Is there a way for other countries to help South Asia? Are there any previous cooperations between these countries?
9. What are some incentives for citizens to stop continuously using antimicrobials?
10. How can countries promote the development of Research and Development (R&D)? How is the funding situation on AMR research?

CLOSING REMARKS

Antimicrobial Resistance (AMR) remains a persistent problem that has affected South Asia since ancient times. Antibiotics which scientists initially called "miracle cure" transformed medical practices by delivering safer and more effective treatments for cancer operations and disease prevention. The excessive use of antibiotics which has continued for decades has led to bacteria and other microorganisms developing resistance at a faster rate than scientists have been able to create new antibiotics.

National Action Plans (NAP) have been implemented by many countries yet the execution of these plans remains deficient which has worsened the existing situation. The public and medical staff lack knowledge about antibiotics which leads them to believe that these drugs can treat viral infections such as the common cold. The combination of high population density and inadequate infrastructure creates an environment that allows AMR to spread throughout the country and beyond its borders.

The case studies of New Delhi and Dhaka, as mentioned, show that local AMR exists in both cities, leading to worldwide effects. The research studies, which examine the NDM-1 resistance gene in New Delhi, show that hospital waste and wastewater and excessive antibiotic use create a major AMR problem which extends throughout the entire city. The city of Dhaka shows that multidrug-resistant fluoroquinolone-resistant typhoid fever develops from antibiotic misuse while genetic mutations occur throughout the city. Both case studies show that AMR begins as a local problem which globalizes when one region experiences easy travel and trade, and environmental spread of infection.

Overall, delegates are expected to identify the root causes of AMR and delve deeper outside of the background guide of the underlying problems within South Asia. The solution requires multiple actions which include strengthening the prescription system and funding public education initiatives while developing better surveillance systems and promoting sustainable farming methods, while ensuring equal access to diagnostic and treatment resources. The issue extends beyond antibiotic usage control because the protection of modern medicine, economic stability, and future development need safeguarding. If the world fails to implement immediate worldwide solutions for antimicrobial resistance control, then society will enter a post-antibiotic era where common infections become deadly again. The current requirement demands that we act together to stop antimicrobial resistance from spreading while all government bodies and health organizations, international bodies, and individual citizens must take responsibility for this duty. Delegates, during conference, it is now your duty to not only save South Asia, but also the entire world.

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